GTT Center Perth

Business Plan

WA's first premium waiting experience for pregnant women having the Glucose Tolerance Test

Prepared by	Grace (Operations Manager) for Anthony Zed
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Status	DRAFT for review. Not yet approved for release.
Version	v0.1
Business name	GTT Center Perth (working name; the name SOLENA is a conditional front-runner, not locked, trademark not cleared)
Trading entity	TO CONFIRM (structure under review with the accountant)
Stage	Pre-launch. No venue, no lease, no signed lab agreement, no trading data.
Currency	All figures in Australian dollars (A$)

Important notice. Every financial figure in this plan is a planning estimate from a financial model. The business has not opened and has no trading history. Items marked "TO CONFIRM" are not yet settled and are listed with the reason in Section 10 and Appendix B. Nothing in this plan has been sent to a lab, landlord, lender or investor.

Table of Contents

1. Executive Summary
2. Value Proposition
3. Perth Market Analysis
4. Services and Pricing
5. Clinical Operations
6. Site and Premises
7. Marketing Strategy
8. Team and Staffing
9. Financial Summary
10. Risks and Mitigations
11. Growth Roadmap and Launch Gates
12. Appendix (A: Sources, B: Superseded figures and how they were resolved)

1. Executive Summary

The business. GTT Center Perth turns the 2 to 2.5 hour wait for the Glucose Tolerance Test (GTT) into a restorative visit. The GTT is a standard pregnancy screening test. Mothers fast, then have blood drawn three times over two hours. Today the wait is spent in a pathology waiting room. At GTT Center Perth the mother books and pays in advance, has her blood drawn on site in a private collection room by our own employed phlebotomists, and uses the time between draws for massage, nails, hair or beauty services in a calm lounge. In the afternoons the same premises sells wellness services and packages to pregnant women who are not having the test.

Why it should work. Demand is not optional: nearly every pregnant woman is screened. Greater Perth has about 26,790 births a year, which is roughly 515 tests a week. Our research found no equivalent venue in WA. This is our own finding, not an independent audit.

The model. Staff are employed (casual or permanent), not subtenants. Up to 18 morning clients a day, Monday to Saturday, from a 07:00 first booking. Clients book 2 to 4 weeks ahead and a waitlist backfills cancellations. Staff are rostered to each morning's confirmed bookings. Lab testing and courier are put to tender between PathWest and Western Diagnostic Pathology (WDP). GTT earns no pathology revenue.

Key numbers card

Measure	Base case (A$8,000 rent assumption)	At the A$10,500 gross rent ceiling
Morning capacity	18 clients a day (9 pairs, 2 chairs)	same
Steady-state operating profit, per month (Month 5 onward)	**+A$35,186.40**	**+A$33,936**
Conservative "GST-stress" downside, per month	+A$23,443	+A$22,306
Break-even, morning clients a day	**8.266** (about 46% of capacity)	8.455
Startup capital (base)	**A$336,198**	A$341,198
Peak funding drawn (with GST timing)	**A$267,771**	A$274,021
Lowest cash point	A$68,427 in Month 1	A$67,177 in Month 1
Payback (planning basis / after GST)	**Month 11** / Month 15	Month 11 / Month 16

Three conditions travel with these numbers. 1. **Rent.** The cost model uses A$8,000 a month. The approved rent budget is A$9,500 to A$10,500 a month. The right-hand column shows the A$10,500 gross ceiling (rent, outgoings and car bays all in). 2. **Fill rate.** The model assumes every released morning slot fills (100%). Each 1% of fill at capacity is worth about A$1,185 a month. At 85% fill, profit is about +A$17.4k a month. At 70% fill the business makes a small loss. This is why the waitlist and the 2 to 4 week booking window are core to the plan. 3. **Cadence.** The 8.266 break-even relies on the 45-minute pair cadence for mornings of 10 or fewer bookings (adopted 2026-10-11). Full mornings run at the 25-minute cadence.

Funding position. The planning need is A$336,198 to A$361,198 (A$251,198 pre-opening capital plus A$85,000 to A$110,000 working capital). Funds on hand are about A$200,000 (as stated in the repository, not re-verified since 2026-07-29). The gap on the peak-funding basis is about A$67,771. The funding route is TO CONFIRM.

Where we are. No venue is secured. No lab agreement is signed. Both are launch gates (Section 11). Hiring and fit-out wait behind them.

2. Value Proposition

2.1 The problem

Today	What mothers get
Wait time	About 2 to 2.5 hours (some sources say up to 3)
Setting	A pathology waiting room, with draws at set times
Condition	Fasted, tired, unable to eat, unable to leave
Care during the wait	None

The test is not optional. The choice that remains is where to have it.

2.2 The solution

Element	What GTT Center Perth provides
Blood collection	On site, in private collection rooms, by GTT's own employed, certified phlebotomists
The wait	Pregnancy massage, facials, nails, hair and brows between draws. A draw never interrupts a service.
Lounge	Venue and lounge access (water, herbal tea, birth plan tablet) is free inside every package
Booking	Book and pay online in advance. Waitlist for popular mornings.
Results	The lab tests the samples and sends results to the mother's doctor. GTT does not diagnose or interpret results.

2.3 What is not in the launch scope

- 3D keepsake ultrasound is a possible Phase 2 idea only. It would be entertainment and keepsake only, never diagnostic.
- Spray tan has been removed from the concept.
- Cafe and retail income is set to A$0 in the financial baseline.

2.4 Honest view of the advantage

Demand is real and non-discretionary, and we are first. The experience itself is not hard to copy. The lasting advantage has to be built from midwife and obstetrician referral relationships, a compliant collection room and a trusted lab partner.

3. Perth Market Analysis

3.1 Market size

Measure	Figure	Note
WA births per year	about 33,570	2024
Greater Perth births per year	about 26,790	2024
Perth GTT tests per year (proxy)	about 26,790	Assumes near-universal screening. Births are used as a stand-in for tests. TO CONFIRM.
Perth GTT tests per week	about 515	26,790 divided by 52
Women diagnosed with gestational diabetes	about 18% nationally	Australian Institute of Health and Welfare, 2023-24 hospital data, as cited in the repository research. National, not WA-specific.

Source for births: ABS Births, Australia 2024, as analysed by KPMG. Figures are preliminary and subject to revision. The next ABS release (2025 data) is scheduled for 6 November 2026.

3.2 Our share of the market

Measure	Figure
Planning capacity	108 morning slots a week (18 a day, 6 days)
Capacity as share of Perth weekly tests	about 21%
Break-even volume as share of Perth weekly tests	about 9.8%, or roughly 1 in 10 Perth GTT mothers

The honest question is not whether the market is big. It is whether we can win about 1 in 10 Perth GTT mothers through referrals and keep every released morning full.

3.3 Referral window

A GTT is usually done at 24 to 28 weeks of pregnancy (earlier for women with risk factors). A valid pathology request form from a GP, obstetrician or midwife is the standard requirement. Each mother therefore has several mornings that suit her, which supports advance booking.

3.4 Competitors

Competitor	What they offer	Position
MIWM (Melbourne)	GTT plus a 60-minute or 2-hour spa treatment. One client at a time. GP-led, clinical feel. No hairdressing. Reported fully booked 3 to 4 weeks ahead.	The only comparable business our research found in Australia. Victoria only.
Pathology collection rooms (PathWest, WDP and others)	The test only. Waiting room.	The current default. They are also potential partners.
Perth day spas	Discounted spa packages average about A$140.	Not GTT-linked. Price reference only.

Competitor	What they offer	Position
WA wellness-during-GTT venue	None found	Our research found no equivalent venue in WA.

MIWM	GTT Center Perth
Single client at a time	Up to 9 pairs of clients a morning, staff rostered to bookings
Spa treatment fixed in the package	Client chooses her services
No hairdressing	Full hairdressing menu
Victoria only	Western Australia

Sources: market-research-findings.md, executive-summary.md, CURRENT-STATE.md (Appendix A).

4. Services and Pricing

4.1 Morning GTT packages

Package	Price	What it includes
GTT Package 1	**A$250**	Venue and lounge free, plus 2 x 30-minute services (fixed)
GTT Package 2	**A$300**	Venue and lounge free, plus a choice of 2 x 45 minutes, 45 + 30 minutes, or 2 x 30 minutes

- No package is sold lounge-only.
- The financial baseline prices every morning client at A$250, which is a deliberately cautious assumption. Package 2 upgrades are upside, not counted.
- The pathology fee is billed separately by the lab. The estimated out-of-pocket cost is A$0 to A$80, pending partner confirmation. TO CONFIRM.

4.2 Afternoon services

Offer	Price	What it is
PM Refresh	**A$185**	75 minutes: massage plus mini facial (A$212 if bought separately)
PM Restore	**A$135**	75 minutes: manicure plus blow-dry (A$150 if bought separately)
Individual services	See 4.3	Available alongside the packages. Marketing leads with the packages.

Afternoon capacity is 10 sessions a day in the financial model. PM package prices were signed off by Anthony on 2026-10-03.

4.3 Indicative individual prices (services menu, locked list)

Service	Duration	Price
Express pregnancy massage	30 min	A$75
Standard pregnancy massage	45 min	A$120
Express glow facial	30 min	A$95
Signature pregnancy facial	45 min	A$130
Classic manicure	30 min	A$55
Gel manicure	45 min	A$75
Brow wax and tint	25 min	A$55

Hair services and further add-ons are on the full menu. All morning services are 45 minutes or under so that they fit inside a service slot. Prices are our own and have not been tested with customers.

4.4 Revenue streams at steady state

Stream	A$ per month	Share
Morning GTT packages (18 clients a day x A$250 x about 26.33 trading days)	118,485.00	about 83%
Afternoon services and packages (10 sessions a day)	24,585.37	about 17%
Total revenue	**143,070.37**	100%

Not counted in the baseline (upside only): cafe and retail, staff downtime fill and early-release savings, Package 2 upgrades, and the Phase 2 keepsake scan.

5. Clinical Operations

5.1 Who does what

Task	GTT Center Perth	Lab partner
Venue, collection rooms, chairs	Provides	Inspects and signs off
Employs the phlebotomists	Yes	Must engage or credential them under its own quality system
Draws blood	Yes	No
Courier and lab testing	No	Yes (winning lab)
Medicare billing for the test	No. GTT earns nothing from pathology.	Yes
Results to the doctor	No	Yes
Diagnoses or interprets results	Never	Not applicable (the doctor does)

Marketing must not say "we perform GTT testing". The phlebotomist's scope is pathology collection only.

5.2 Scheduling standard (adopted 2026-10-11)

Item	Standard
First booking	07:00, both chairs start together
Chairs and phlebotomists	2 chairs, 2 in-house phlebotomists on every morning with any GTT booking
Full mornings (15 or more bookings on a weekday, 17 or more on a Saturday)	25-minute pair cadence, 9 pairs, **18 clients**, 8 therapists. Last first-draw 10:20.
Quiet mornings (10 or fewer bookings)	**45-minute pair cadence**, up to 10 clients, 4 therapists. Last first-draw 10:00.
Last first-draw limit	No later than 10:30 (lab guidance)
Booking	2 to 4 weeks ahead, prepaid, plus a waitlist
Trading days	Monday to Saturday

The 25-minute and 45-minute schedules were checked by the repository's scheduling solver (run 2026-10-11) with zero chair clashes. The 45-minute cadence applies to low-volume mornings only. Applied to every morning it would cap capacity at about 10 clients a day.

5.3 One client, one visit

Clock	Step
Arrival	Arrives fasted. Check-in, request form, Medicare card, two identity checks.
0 minutes	Draw 1 (fasting, about 5 minutes), then the glucose drink

Clock	Step
After Draw 1	Service 1
60 minutes exactly	Draw 2
After Draw 2	Service 2
120 minutes exactly	Draw 3. The mother may leave only after this. Snack offered at once.

The client stays on site and seated or escorted throughout. Services are low-activity only. Specimens are labelled at the chair, stored in a medical fridge at 2 to 8 degrees C (kept apart from food), and handed to the courier once, at the end of the morning.

5.4 Lab tender (dual-lab strategy, locked 2026-10-10)

GTT employs two certified in-house phlebotomists. Courier and lab analysis are put to tender between PathWest and WDP for the best per-test fee schedule. Neither lab has an exclusive commitment. Clinipath is an optional third bidder only if it replies.

Criterion	PathWest	WDP
Per-test fee	TO CONFIRM	TO CONFIRM
Courier pickup	Confirmed 2026-09-01 for the correct tubes. Times TO CONFIRM.	Feasible with the correct tubes. Overnight storage conditional. No fixed cutoff.
Tubes and bags	Free if volume is large	TO CONFIRM
Glucose drink	Not supplied	TO CONFIRM
Turnaround, result delivery, minimum term	TO CONFIRM	TO CONFIRM
Approved Collection Centre (ACC) arrangement	TO CONFIRM	TO CONFIRM

Caveat that is not yet solved. Only the pathology partner can hold the Commonwealth ACC approval for a collection room, and two competing labs cannot both hold it for the same room. Options to test are one ACC-holding lab per room (we plan two rooms), a separate arrangement per room or per lab, or one lab holding it while the other does courier and analysis only if its rules allow. This must be verified with both labs and an adviser before it is relied on. Nothing is signed and neither lab has quoted a fee.

5.5 Compliance

Area	Position	Status
Collection room standard	National Pathology Accreditation Advisory Council guidelines for Approved Pathology Collection Centres (third edition, 2013). Applies to the blood room.	Design stage
Lab accreditation	Covered by the partner's accreditation. GTT needs none in its own name.	Confirmed in repository
Phlebotomist qualification	Certificate III in Pathology Collection (code HLT37525). Phlebotomists are not AHPRA-registered.	Hiring on hold
Beauty and nail services	WA skin penetration regulations and Code apply to these services only	To be met at fit-out

Area	Position	Status
Privacy	Privacy Act 1988 (Cth). Health data is sensitive. GTT does not hold results.	Policy in draft
Consent form and privacy policy	Drafts. Solicitor review required before any patient data is collected.	Open
Referral law	Health Insurance Act 1973 (Cth) s129AA may touch referral perks for doctors. Solicitor advice needed.	Open
Insurance	No broker quote yet. Public liability and professional indemnity are modelled only. Clinical-liability and cyber cover unresolved.	Open
WA Health licence for a blood collection room	Not established in the repository	Verify with the partner and WA Health
Emergency procedures	Emergency plan drafted. Hypoglycaemia and needle-stick procedures still to be written.	Open

Items in this table drawn from the repository are general information. Verify each with the adviser or regulator before external reliance.

6. Site and Premises

6.1 Requirements (locked 2026-10-10)

Item	Requirement
Floor area	**257 to 290 sqm** net usable. 257 sqm is the minimum.
Blood collection rooms	**2 dedicated rooms**, solid walls, one lockable door each (essential for clinical flow and to prevent bottlenecks)
Rent budget	**A$9,500 to A$10,500 a month gross** (base rent plus estimated outgoings, excluding GST). A$10,500 is the hard all-in ceiling.
Level and access	Ground floor only, accessible path of travel, accessible toilet, doors at least 850 mm clear in clinical rooms
Parking	8 to 10 or more client bays and 3 to 5 staff bays, well lit
Services	3-phase power, plumbing for hands-free basins in each collection room and at treatment bays, working ventilation and air conditioning, good internet
Hours	Monday to Saturday. Clients arrive from about 06:45.
Permitted use	Health, beauty and wellness services, including pathology specimen collection and a small refreshment counter

6.2 Lease terms sought

Term	Seeking
Initial term and option	3 years plus a 3-year option
Rent review	Fixed percentage or CPI, no ratchet clause
Rent-free	3 to 6 months during fit-out
Collection rooms	Lease must allow us to licence or sublet one or both rooms to an accredited pathology provider, and to change provider later
Make-good	Negotiated. Avoid full reinstatement of blood room plumbing.
Personal guarantee	Cap at 6 months rent. Tenant entity TO CONFIRM.
Lease bond and legal	About A$19,600 to A$27,600 (modelled)

No landlord contribution is counted in the financial model.

6.3 Target areas

Priority	Area	Why
1	Osborne Park	Freeway access, strong parking stock
2	Joondalup	Northern corridor hub, near Joondalup Health Campus

Priority	Area	Why
3	Cannington and East Cannington	Albany Highway and Roe Highway, near Bentley Health Campus
4	Myaree and Murdoch	Kwinana Freeway and Leach Highway, Murdoch health precinct

6.4 Shortlist leaders (not verified as available today)

Property	Size	Rent	Notes
6/325 Harborne St, Osborne Park	268 sqm	A$55,000 a year plus A$23,785 outgoings (about A$6,565 a month plus GST)	Blank shell. Plumbing unknown. Re-checked 2026-08-14.
25 Mills St, Cannington	up to 302 sqm	Not disclosed	Existing medical fit-out near Bentley Hospital. No listing link in the repository. Needs verifying.

Status. No venue, no lease, no heads of agreement. The collection-room specification must be obtained in writing from both labs before any lease is signed. An architect will be engaged once a site is chosen. Fit-out has not been repriced for the larger footprint.

7. Marketing Strategy

7.1 Principle

The main customer channel for the morning GTT is the referring practice, not paid advertising. Every pregnant woman already has a GTT referral. The task is to make the practice aware that a better place to have it exists. The afternoon customer is different: she is discretionary and finds us through Instagram, Google and word of mouth.

7.2 Morning GTT: referral network

Element	Plan
First-wave targets	22 named practices already identified: 12 private midwifery practices and 10 obstetric and gynaecology clinics. None contacted yet.
Why these	Independent enough to recommend a collection venue. Hospital-based public clinics tend to have a default pathway, so the public-sector channel is lower priority.
Test batch	First 3 midwifery practices with a direct personal contact. Expand to the remaining 19 only after all 3 have a recorded outcome and at least 1 gives a genuine "yes, we would refer".
Approach	Personal visit or call first, then a premium referral card, a one-page practice information sheet, and an invitation for the practice's staff to experience the service once open
GP channel	No GP practice is named in the repository. Method: target bulk-billing GP clinics with high antenatal volume near the confirmed venue.
Timing	Pre-launch activity that depends on a confirmed venue address

Each visit must end in one recorded outcome: the practice displays cards, accepts a complimentary visit, or declines. A visit with no recorded outcome does not count.

Referral metrics (framework, not a forecast):

Metric	Definition
Outreach completion	Practices visited out of 22
Adoption rate	Practices displaying cards out of practices visited
Referral share	Morning bookings tagged to a practice out of all morning bookings
Bookings per practice per month	Identifies top referrers for quarterly check-ins

Tracking uses the "How did you hear about us?" field in the booking system. It is a named Venue Manager responsibility from week 1 of opening.

Referral law. Offers to practices are limited to a complimentary visit and information. Any formal referral credit or partner pricing for health practitioners needs solicitor advice first (see Section 5.5).

7.3 Online booking and waitlist

Element	Plan
Website	Waitlist form (name, email, mobile, suburb, due date, services of interest, how they heard of us), how-it-works page, service menu with prices, embedded online booking, FAQ (what a GTT is, fasting instructions, what to bring)
Booking	Online, prepaid, 2 to 4 weeks ahead. Roster published 1 to 2 weeks ahead from confirmed bookings.
Waitlist role	Backfills cancellations and lets us hold a morning at 10 bookings instead of opening a half-used full roster
Waitlist targets	100 sign-ups by week 13 and 300 or more by week 18 (repository planning target)
Waitlist launch	48-hour priority booking window for waitlist members before public launch
Email	Welcome email, an educational email about 3 weeks later, then a launch notice

7.4 Brand and community channels

Channel	Use
Instagram	2 to 3 posts a week over a 12-week pre-launch calendar: the problem, the solution, service spotlights, the lounge, a countdown. Educational posts make no medical claims.
Pregnancy communities	Value-first participation in Perth parenting groups and sites. No promotion in groups that ban advertising.
Google Business Profile	Created once the venue address is confirmed. Reply to every review within 24 hours.
Search	Priority terms: "GTT test Perth", "gestational diabetes test Perth", "GTT waiting Perth". Pathology providers show information pages only.
Media	Perth press, parenting blogs and community media, with a "first in WA" angle once the venue is real

7.5 Afternoon demand

Element	Plan
Offer	Lead with PM Refresh and PM Restore. Individual services stay visible as a secondary option.
Partners	Prenatal yoga studios, women's health physiotherapists, antenatal class providers, doulas, maternity retailers and pregnancy photographers
Group bookings	Antenatal cohorts, baby-shower groups, mother and daughter visits
Paid search	Separate afternoon campaign for pregnancy wellness terms
Retention	Post-visit review request and a re-booking nudge

Afternoon volume is a modelled assumption of 10 sessions a day. It has no booking data behind it yet. A postnatal offer is an upside idea only.

8. Team and Staffing

8.1 Principle: demand-responsive rostering (Anthony, 2026-10-11)

Staff are rostered to the number of clients booked that morning. Clients book weeks ahead and a waitlist is kept. This does not change steady-state profit (18 clients a day needs the full roster). It cuts the ramp-up cost: payroll for Months 1 to 3 falls from A$280,787 to A$214,844.

8.2 Roles

Role	Number	Basis	Notes
Venue Manager	1	Monday to Friday, fixed	Not yet hired. First responder, first aider, EpiPen holder, fire warden. Hire is gated behind the lab agreement and the venue. Saturday duties go to an experienced rostered staff member.
Phlebotomists (in-house, certified)	2 on shift, 4 on the books	Casual, review at 3 to 6 months	Chair A and Chair B, 07:00 to 13:00. Relief pool because nobody else can cover phlebotomy.
Morning therapists (massage and beauty pool, nails, hair)	4 on quiet mornings, 8 on full mornings	Casual, dual-qualified where possible	8 = 4 massage and beauty pool, 2 nails, 2 hair
Afternoon therapists	4 roles (massage, hair, nails, beauty)	Casual, paid for hours worked	Cross-shift qualified with the morning pool
Reception	No separate role	Venue Manager covers mornings. Rostered afternoon staff cover check-in.	Dedicated PM reception role removed 2026-08-21

Awards: Hair and Beauty Industry Award (MA000005) and Health Professionals and Support Services Award (MA000027). Pay-rate clauses are to be confirmed with Fair Work. Super is costed at 12% and workers compensation at 1.7% (a placeholder rate).

8.3 Morning roster rule

Confirmed bookings that morning	Phlebotomists	Therapists	Pair cadence	Weekday cost incl. on-costs	Rule
0	0	0	n/a	A$0	No morning clinical roster
1	2	1	n/a	A$724.84	Floor: 2 phlebotomists whenever any GTT client is booked
2 to 5	2	1 to 4	45 min	A$724.84 to A$1,482.90	Avoid. Steer bookings to fill the morning.
6 to 10	2	4	45 min	A$1,482.90	Target band

Confirmed bookings that morning	Phlebotomists	Therapists	Pair cadence	Weekday cost incl. on-costs	Rule
11 to 14	2	8	25 min	A$2,496.78	Avoid. Costliest per client. Hold at 10 and offer the waitlisted client another morning.
15 to 18	2	8	25 min	A$2,496.78	Full roster only at 15 or more bookings on a weekday or 17 or more on a Saturday

Saturday costs 1.5 times the weekday figure. The Venue Manager is fixed and not in these figures.

8.4 Hiring sequence

Recruitment documents are prepared. Hiring starts only after the lab agreement and venue are secured, then the Venue Manager first, then phlebotomists and treatment staff, then training and test runs. This compresses the hiring window once the gates clear (see Section 10).

9. Financial Summary

Full statements are in the companion documents, prepared in parallel by the finance workstream:

Document	Contents
PROFIT-AND-LOSS.md	Monthly and annual profit and loss
CASHFLOW-FORECAST.md	Monthly cash, funding and GST timing
3-WAY-FINANCIAL-MODEL-AUD.md	Profit and loss, cash flow and balance sheet, with every assumption and source

9.1 Steady state (Month 5 onward, 18 clients a day, A$8,000 rent assumption)

Line	A$ per month	A$ per year
Revenue	143,070.37	1,716,844.44
Payroll (including super and workers compensation)	93,595.63	
Other costs (direct costs, rent budget A$8,000, insurance, administration)	14,288.34	
Total costs	**107,883.97**	
Operating profit	**+35,186.40**	**+422,236.80**
Profit margin	about 24.6%	

9.2 Profit under different rent and tax cases

Case	Net per month	Break-even (clients a day)	Peak funding	Payback (planning / after GST)
Base, A$8,000 rent	+A$35,186	8.266	A$267,771	Month 11 / Month 15
GST-stress downside	+A$23,443	9.255	not separately modelled	
A$10,500 gross ceiling, all-in	+A$33,936 (GST-stress +A$22,306)	8.455	A$274,021	Month 11 / Month 16
A$10,500 plus separate utilities and cleaning (prudent)	+A$32,686 (GST-stress +A$21,170)	8.645	A$275,271	Month 12 / Month 17

9.3 First-year ramp

Revenue is 43%, 64%, 79% and 93% of steady state in Months 1 to 4, then 100%. The origin of this ramp curve is not documented in the repository.

Month	Average morning clients a day	Net (A$)
1	7.74	-16,573
2	11.52	+7,783
3	14.22	+19,292

Month	Average morning clients a day	Net (A$)
4	16.74	+30,022
5 to 12 (each)	18.00	+35,186
Year 1 net		**A$322,015** (A$196,609 after GST timing)

9.4 Startup capital and funding

Item	A$
Pre-opening capital (approved in principle 2026-08-10)	251,198
Opening working capital reserve	85,000 (up to 110,000)
Startup capital, base	**336,198**
Startup capital at the A$10,500 rent cases	341,198
Peak funding actually drawn (with GST timing)	267,771
Lowest cash point	68,427 (Month 1)
Funds on hand (as stated in the repository)	about 200,000
Gap on the peak-funding basis	about 67,771
Gap on the combined planning case (336,198 to 361,198)	about 136,198 to 161,198

Of the pre-opening capital, fit-out and construction is the largest group (A$139,678 in the base case). If bookings stall at Month 1 volume, the A$85,000 reserve covers about 4 full months. The repository holds several unreconciled startup ranges, including an adopted range of A$292,335 to A$594,900. The accountant and a quantity surveyor are to confirm. Rent during the fit-out period is not in the base budget.

9.5 Sensitivities

Scenario	Net per month
Base (100% fill)	+A$35,186
85% fill	about +A$17.4k
70% fill	small loss
Volume down 20%	+A$19,573
AM price down 10% (A$225)	+A$20,879
Each A$10 per client of lab fee	about A$4,739 less at capacity

Lab fees are assumed to be A$0 per client in the model because neither lab has quoted. This is an open assumption.

9.6 Tax and structure

- GST: a two-tier invoice is proposed, with a clinical phlebotomy component possibly GST-free as a health service (GST Act s38-10) and the lounge component taxable. The section has conditions on who supplies the service and GTT is not a pathology provider, so this is not settled. The accountant or an ATO ruling is to confirm.

- A proposed 30% minimum tax on discretionary trusts from 1 July 2028 is an exposure draft, not law. Accountant review is needed.
- Trading entity: TO CONFIRM.

10. Risks and Mitigations

#	Risk	Level	Mitigation
1	No venue secured. Blocks hiring, fit-out and everything after.	Critical	Two shortlist candidates. Verify rent and availability. Inspect.
2	No signed lab agreement. Fees from both labs TO CONFIRM. ACC arrangement across two labs unresolved.	High	Run the tender and get written fee schedules from both. Take adviser advice on ACC. Clinipath only if it replies.
3	Funding gap. Planning need A$336k to A$361k against about A$200k on hand. Peak funding drawn A$267,771.	High	Founder funding decision. Accountant. Staged fit-out. TO CONFIRM.
4	Demand unproven. Fill is assumed 100%. 85% fill gives about +A$17.4k a month and 70% makes a loss.	High	Referral network and waitlist from day one. Book 2 to 4 weeks ahead. Steer mornings to 10. Track fill weekly against an 85% warning line. Cut released slots before adding mornings.
5	Cost model uses A$8,000 rent but budget is A$9,500 to A$10,500.	Medium	Net at the new rent is about +A$32,686 to +A$33,936 a month, shown beside the headline.
6	Courier cut-off time unconfirmed. Documents show 11:30 and 12:30, neither sourced.	Medium-High	Confirm a pickup window with each lab in writing before locking the schedule.
7	Collection-room compliance and cost. Architect not engaged.	Medium	Brief an architect after the site is picked. Confirm the specification with the partner.
8	Insurance not yet quoted, including the clinical line.	Medium-High	Obtain a broker quote before any commitment. Insurance is a launch gate.
9	Safety cover depends on one not-yet-hired Venue Manager.	Medium-High	Cross-train a backup responder. The venue does not open without this cover.
10	Referral law (s129AA) and the draft consent and privacy documents.	Medium	Solicitor review before any patient or waitlist data is collected and before any practice offer.
11	GST treatment of the proposed two-tier invoice is unsettled.	Medium	Accountant or ATO ruling. GST-stress downside is +A$23,443 a month.
12	Proposed 2028 minimum tax on discretionary trusts.	Low-Medium	Annual accountant review of the entity structure.
13	Hiring window compresses once the gates clear, because recruitment cannot start earlier.	Medium	Job documents are pre-built so recruitment starts the day the gates clear.
14	Copycat entering WA.	Low-Medium	First-mover position, referral relationships, brand, and a compliant collection room.
15	Clinical figures (18% diagnosis rate, 2 to 2.5 hour test length) not re-checked against a current Australian clinical guideline.	Low-Medium	Verify against a current clinical source before external use.

Risks 1 to 3 are one single point of failure: no site and no lab partner means no revenue. The order of fixing matters more than anything else in this table.

11. Growth Roadmap and Launch Gates

No calendar dates are set. The venture holds launch undated until the gates clear.

11.1 Stages

Stage	What happens	Gate	Status
0	Venue search. Lab tender talks. Marketing waitlist can start early.	none	In progress
Gate A	Lab agreement(s) in place from the tender, with written terms and an ACC arrangement	Written terms	Open
Gate B	Venue confirmed, lease reviewed by a solicitor	Site and lease terms	Open
1	Venue Manager hire. Lease or heads of agreement. Architect and fit-out quotes.	A and B	Not started
2	Phlebotomist and treatment staff recruitment and contracts	Venue Manager in place	Not started
3	Partner site inspection, training, test runs, soft open for the waitlist, public launch	Fit-out done, staff hired	Not started
Go-live	First confirmed customer booking. The venture moves from standby to active.		Not started

11.2 Launch gate checklist

Gate	Condition	Status
G1	Signed lab agreement(s): fees, courier, term, ACC arrangement	Open
G2	Venue secured (257 to 290 sqm, 2 collection rooms, rent within A$9,500 to A$10,500) and lease reviewed	Open
G3	Each lab used signs off the room and credentials our phlebotomists	Not started
G4	Insurance bound, including the clinical line	Open
G5	Venue Manager hired (first aid, EpiPen, fire warden)	On hold
G6	Phlebotomists, relief pool and casual treatment pool hired	On hold
G7	Consent, privacy and referral-law advice cleared	Open
G8	First confirmed booking activates the venture	Not started
G9	Booking and waitlist system live: prepaid booking 2 to 4 weeks ahead, waitlist, roster published 1 to 2 weeks ahead	Not started
G10	Weekly fill-rate tracking against the 85% warning line	Not started

11.3 Decisions needed

#	Decision	Owner
1	Run the dual-lab tender: written per-test fees, courier cutoffs, term and ACC arrangement	Anthony, Reed
2	Pick a site to verify first (Harborne St or Mills St)	Anthony
3	Decide how to cover the funding gap and brief the accountant on entity structure	Anthony
4	Appoint an architect once a site is chosen	Anthony
5	Accountant: confirm the two-tier GST invoice and whether an ATO ruling is needed	Anthony
6	Adviser and both labs: how ACC approval works with two competing labs	Anthony, Reed
7	Build the booking and waitlist system and weekly fill tracking	Ivy

11.4 Growth options (not in the baseline)

Option	Status
Fill rate above plan, Package 2 upgrades, downtime fill and early release	Upside, unproven
Postnatal offer in the afternoon	Idea only
3D keepsake ultrasound	Possible Phase 2, keepsake and entertainment only, never diagnostic
Third collection chair or room	Undecided. The two-room design changes the growth path.

12. Appendix

Appendix A. Sources

Approved pack (DRAFT, v0.3 to v0.4, 2026-10-10 and 2026-10-11)

Document	Used for
EXECUTIVE-PITCH-DECK.md (v0.4)	Market, pricing, unit economics, risks, roadmap, partnership status
CLINICAL-OPERATIONS-PLAN.md (v0.4)	Scheduling standard, patient flow, compliance, lab tender
FINANCIALS/3-WAY-FINANCIAL-MODEL-AUD.md (v0.4)	All financial figures
LEASING/COMMERCIAL-SITE-BRIEF.md (v0.3)	Premises, lease terms, target areas
OPERATIONS/GTT-LAUNCH-READINESS-TRACKER.md (v0.4)	Launch gates, bidder status
FINANCIALS/PROFIT-AND-LOSS.md and CASHFLOW-FORECAST.md	Companion statements (in preparation)

Repository (clawanthonyzed/gtt-center-perth, branch main)

- CURRENT-STATE.md
- DECISION-LOG.md
- executive-summary.md
- market-research-findings.md
- MARKET-RESEARCH-BIRTHS-GDM-REFERRAL-2026.md
- services-pricing-locked.md
- PM-PACKAGES.md
- poppy-marketing.md
- afternoon-marketing-plan.md
- referral-partnership-plan.md
- risk-register.md
- STAFF-PROFILES.md
- AM-DEMAND-DRIVEN-STAFFING-TIERS-2026-09.md
- PERTH-PROPERTY-SHORTLIST.md
- business-plan.md (older version 5.3, superseded where it differs; see Appendix B)

External sources cited by the repository: ABS Births, Australia 2024 (as analysed by KPMG; preliminary); Australian Institute of Health and Welfare, gestational diabetes (2023-24); National Pathology Accreditation Advisory Council, Guidelines for Approved Pathology Collection Centres (third edition, 2013); MIWM Melbourne (miwm.com.au). External regulatory points in this plan are general information and must be verified with the relevant adviser or regulator (WA Health, AHPRA where applicable, Fair Work, the ATO) before external reliance.

Appendix B. Superseded figures and how they were resolved

Where the older business plan (version 5.3) disagrees with the current pack and CURRENT-STATE, the newer figure is used.

Topic	Older business plan	Used in this plan	Reason
Steady-state revenue and costs	A$154,711 revenue, A$110,545 costs	A$143,070.37 revenue, A$107,883.97 costs	PM capacity locked at 10 sessions a day (2026-09-19)
Monthly operating result	+A$44,166	+A$35,186.40	Same change. Headline locked by Anthony 2026-10-10.
Break-even	About 11.29 clients a day	8.266 (locked 2026-10-11)	Staff rostered to bookings, 45-minute cadence adopted
Even older baseline	+A$25,087.07 on 570 visits a month	Not used	Historical 10-client model
Afternoon pricing	Individual services at about A$95 a visit	PM Refresh A$185 and PM Restore A$135	PM packages signed off 2026-10-03
Pathology model	Final phlebotomist and lab arrangement unresolved	2 in-house phlebotomists; courier and lab tendered between PathWest and WDP	Decided 2026-09-19 and 2026-10-10
Business entity	A named trust trading as GTT Center Perth	TO CONFIRM	Structure under review (decision log 2026-09-04)
Premises and rent	Smaller footprint, A$8,000 rent	257 to 290 sqm, 2 collection rooms, A$9,500 to A$10,500	Locked 2026-10-10
Birth counts	Older repository files show about 18,000 WA and 14,400 Perth births	About 33,570 WA and 26,790 Greater Perth	Corrected 2026-07-31 to ABS 2024 via KPMG

Open items carried into this plan: lab fees and the ACC arrangement for two labs; the 100% fill assumption; the rent difference between the cost model and the budget; startup capital ranges; the GST treatment; funds on hand; and the trading entity.